

Global Tuberculosis Burden & Treatment Gaps

Five-minute pitch + three-minute Q&A preparation

PRESENTATION SCRIPT

A decision-support tool for understanding where tuberculosis burden is concentrated, how trends have changed, and where diagnosis-and-treatment coverage remains incomplete.

Name	[Student Name]
Major	Master of Science in Business Analytics
Department	Suliman S. Olayan School of Business
Date	23 June 2026

Project snapshot Analytical period 2000–2024 · 217 countries/territories · six WHO regions · age/sex cross-section for 2024 · reproducible source manifest and validation checks.

Prepared as an individual master's-level healthcare analytics project

PITCH MAP

Five-minute delivery plan

Target speaking time: approximately 5:00. The script is designed for a calm pace, with one main message per slide. The presentation should sound conversational rather than memorized.

Slide	Purpose	Target time	Length
1	Opening	0:00–0:40	71 words
2	Problem and objective	0:40–1:25	86 words
3	Data and method	1:25–2:10	72 words
4	Global trend	2:10–3:00	74 words
5	Geography and gaps	3:00–3:50	73 words
6	Demographics and subtypes	3:50–4:35	83 words
7	Recommendations and close	4:35–5:00	81 words

Three messages to repeat

- The dashboard combines burden, rates and coverage instead of ranking countries on one measure.
- The notification difference is a surveillance signal, not a verified count of untreated people.
- The final recommendations depend on geography, demographics, TB–HIV co-burden and drug resistance.

Delivery cue Pause after each headline number. Point to the visual before explaining it. Do not read chart labels line by line.

SCRIPT · PART 1

Opening, problem and method

Slide 1 · Opening | 0:00–0:40

Tuberculosis is preventable and curable, yet it remains one of the world's largest infectious-disease burdens. My project turns WHO-derived country-level estimates into a Streamlit decision-support dashboard. In the reproducible 2024 snapshot, about 10.55 million people developed TB, 8.33 million new and relapse cases were notified, and calculated diagnosis-and-treatment coverage was 78.9%. The purpose is to show not only where TB is high, but where the health-system response may still be incomplete.

Emphasis Say “reproducible 2024 snapshot” before the numbers so the audience understands these are the dashboard aggregates.

Slide 2 · Problem and objective | 0:40–1:25

The analytical problem is the gap between estimated disease burden and the people who appear in routine services. I calculate the notification difference as estimated incident cases minus notified new and relapse cases. It is deliberately labelled as an estimate and a surveillance signal, because it does not prove that every person in the difference was untreated. The dashboard answers four practical questions: where burden is highest, how trends are changing, who and which subtypes are most affected, and where a healthcare system should investigate first.

Do not overclaim Avoid saying that all 2.22 million people were untreated. Say “estimated notification difference” or “coverage signal.”

Slide 3 · Data and method | 1:25–2:10

The workflow is reproducible from raw input to dashboard. I retained the raw snapshots, standardized identifiers and numeric fields in pandas, merged the indicators into one country-year model, and recalculated global and regional rates from the country data. Automated checks confirm unique country-year records, the 2000-to-2024 range, non-negative burden values and male/female age-sex data. The final package contains the code, raw and processed files, a data dictionary, source hashes, and validation output.

Global trend and geographic priorities

Slide 4 • Global trend | 2:10–3:00

The core time series shows substantial improvement. Notifications more than doubled since 2000, and calculated coverage rose from 32.4% to 78.9%. However, the 2024 snapshot still contains a 2.22 million difference between estimated burden and notified cases. The 2020 disruption is especially important: notifications fell by 18.1% from 2019 while estimated incidence moved much less. This shows that service access and reporting can deteriorate rapidly even when the underlying disease burden does not disappear.

Slide 5 • Geography and gaps | 3:00–3:50

Geography changes the interpretation. Indonesia had the largest absolute notification difference, about 241 thousand, while India had the largest total burden but calculated coverage above 92%. Myanmar illustrates a different risk: a smaller absolute burden, but coverage below 44%. At regional level, the Western Pacific had the largest absolute notification difference. The dashboard therefore avoids ranking countries on one indicator alone and encourages users to combine absolute volume, population rate and service coverage.

Transition sentence

Use this line “The global trend tells us whether progress is occurring; the geography tells us where the next unit of effort may have the greatest value.”

Numbers to remember

Finding	Value
2024 estimated incident cases	10.55 million
2024 notified new and relapse cases	8.33 million
2024 calculated coverage	78.9%
2024 estimated notification difference	2.22 million
2019 → 2020 notification change	–18.1%
Largest country-level difference	Indonesia · about 241,000

SCRIPT · PART 3

Demographics, recommendations and close

Slide 6 • Demographics and subtypes | 3:50–4:35

The age-sex analysis adds a population lens. Estimated male incidence was about one and a half times female incidence, with the largest adult burden in ages 25 to 54. The subtype analysis adds a clinical-programme lens. HIV-associated TB was concentrated in sub-Saharan Africa, especially South Africa, while drug-resistant TB followed a different pattern led by India and several European and Asian settings. These are not interchangeable problems: TB–HIV requires integrated testing and treatment pathways, while resistance requires rapid susceptibility testing and effective regimens.

Slide 7 • Recommendations and close | 4:35–5:00

The dashboard leads to four actions: prioritize high-volume detection gaps; protect TB services during disruption; integrate TB–HIV and drug-resistance pathways according to the local pattern; and tailor outreach while governing the data carefully. The value of the project is the sequence: find where burden and coverage gaps are concentrated, target the population and programme response, and act with measurable interventions. The app, data, code, manual, report and presentation are ready for deployment; the final step is publishing through my Streamlit account.

Final sentence

Close strongly “This dashboard helps a healthcare decision-maker move from finding the problem, to targeting the response, to acting with measurable evidence.”

Presentation-day checklist

- Open the published dashboard and pitch deck before class; keep the dashboard on the executive overview.
- Replace [Student Name] and paste the final Streamlit URL in the deck, report and README.
- Practice once at normal pace and once with a strict five-minute timer.
- During Q&A, answer the question first, then provide one supporting number or limitation.